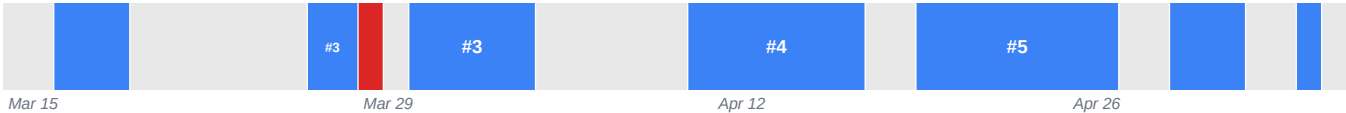


Patient ID: EGPatient ID: EGPeriod: March 15 to May 6, 2024Report Date: May 7, 2024Coverage: Bed: 915/1245h (73.5%)Chair: 670/1245h (53.8%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 53 days from Mar 15 to May 06

■ HR ■ Breathing



Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
39	45h	High Breathing	<1	100%

Major Findings: Stable baseline

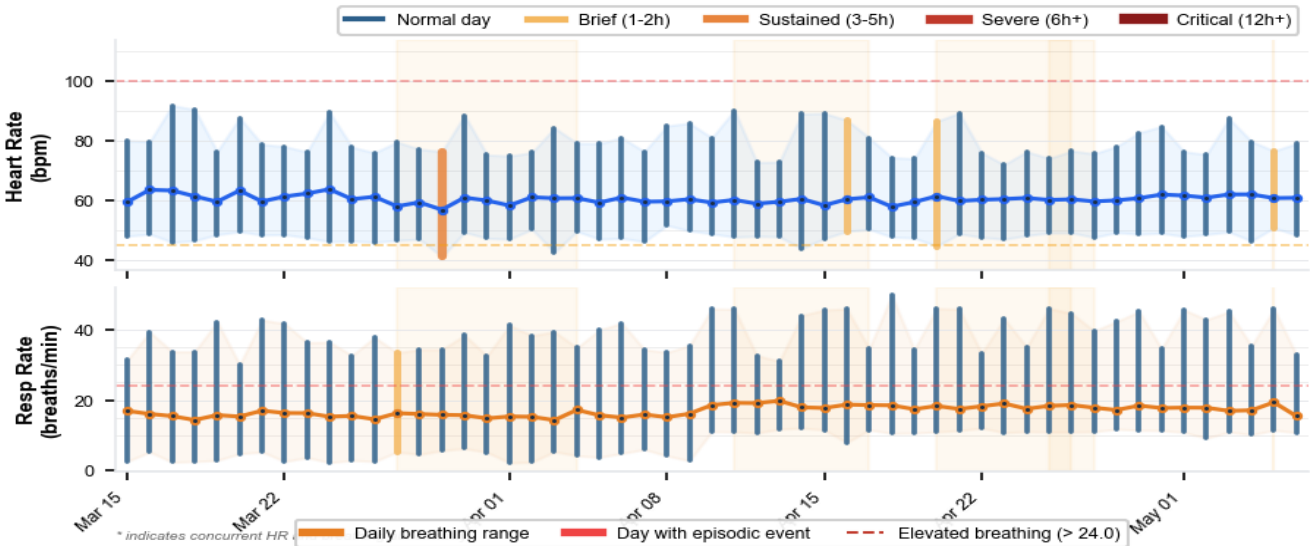
#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/d ay	Average	Date
1	High Breathing	46 brpm	2h	1h	<1	18 brpm	Apr 25 to Apr 27
2	High Breathing	32 brpm	1h	1h	1	19 brpm	May 05
3	Elevated Breathing	40 brpm	14h	2h	1	16 brpm	Mar 27 to Apr 04
4	Elevated Breathing	46 brpm	14h	2h	2	19 brpm	Apr 11 to Apr 17
5	Elevated Breathing	46 brpm	7h	1h	1	18 brpm	Apr 20 to Apr 26
6	Elevated Breathing	46 brpm	2h	1h	2	19 brpm	May 05
7	Low Heart Rate (brief)	41 bpm	3h	2h	2	46 bpm	Mar 29

+ 1 additional condition(s) across Very High Breathing (brief); details in monitoring data.

RR values outside physiologic range (6–60 breaths/min).

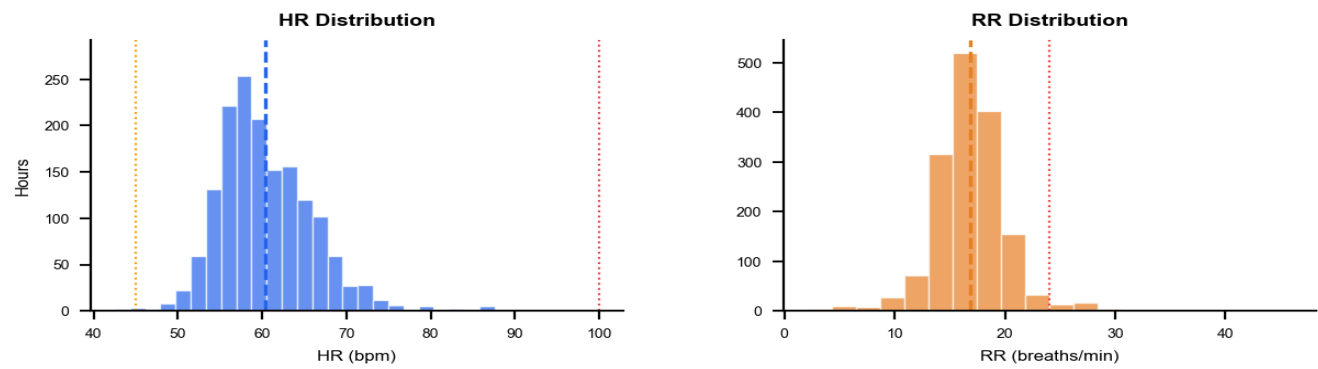
Clinical Pattern Observations:

- 1. Clustered pattern: Episodic events on 5 of 53 days (9%).
- 2. Daytime pattern: Episodes concentrated during waking hours.

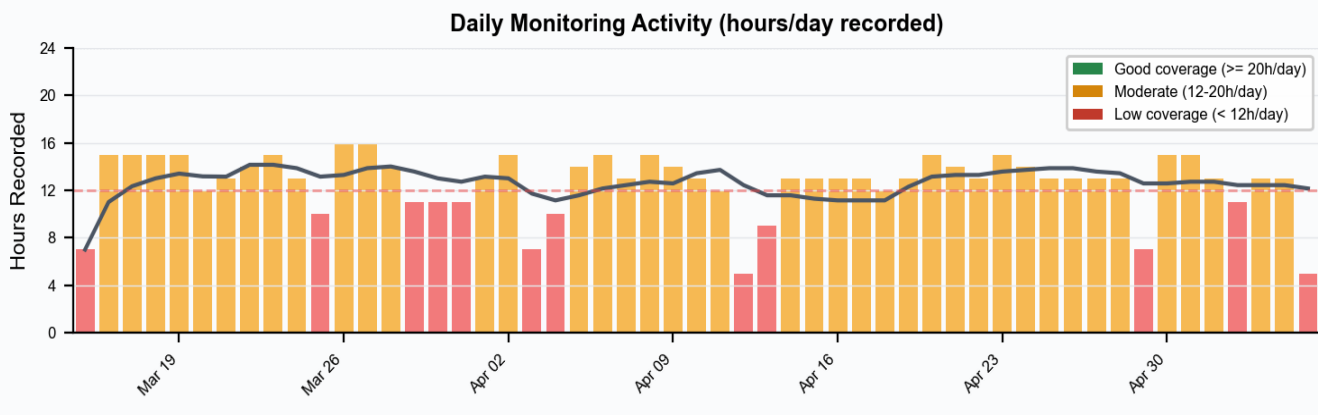


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

■ Very Low HR < 40 bpm	■ Low HR < 45 bpm	■ Elevated HR > 95 bpm	■ High HR > 100 bpm
■ Very High HR > 110 bpm	■ Elevated Breathing > 24 brpm	■ High Breathing > 30 brpm	■ Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.